

Infection Prevention & Control Policy

SECTIONS 1-2: PURPOSE & SCOPE

1 PURPOSE

This policy establishes clear infection prevention and control (IPC) standards across Chosanta Management to protect residents, staff, visitors, and contractors from the transmission of infectious diseases, in accordance with HIQA National Standards, the Health Act 2007, and HSE best practice guidelines.

- ✓ Minimise risk of infection in care settings
- ✓ Promote hygiene and sanitation
- ✓ Support outbreak management
- ✓ Ensure staff understand their roles in maintaining infection control at all times

2 SCOPE

This policy applies to all locations, staff, residents, contractors, agency personnel, and visitors. It encompasses residential units, common areas, kitchens, laundry, bathrooms, medical care delivery, transport, and external activities.

Relevant to all infectious risks, including:

- ✓ Respiratory illnesses (e.g. COVID-19, influenza)
- ✓ Gastrointestinal infections (e.g. Norovirus)
- ✓ Bloodborne viruses (e.g. Hepatitis B/C)
- ✓ Antibiotic-resistant organisms (e.g. MRSA, CPE)

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SECTIONS 3-4: RESPONSIBILITIES & KEY MEASURES

3 RESPONSIBILITIES

- ✓ **Registered Provider:** Ensure the organisation has effective IPC systems and resources; oversee compliance with HIQA standards
- ✓ **Person in Charge (PIC):** Implement IPC policy daily; ensure training; monitor infection risks and outbreaks; report to HIQA
- ✓ **Staff Members:** Follow IPC protocols including hand hygiene, PPE, and safe waste disposal; report symptoms or breaches immediately
- ✓ **Residents & Visitors:** Maintain personal hygiene; comply with outbreak precautions and visitor policies

4 KEY MEASURES

4.1 Hand Hygiene:

- ✓ Hand washing facilities with soap and water at key locations; alcohol-based sanitiser in communal areas
- ✓ Staff and residents educated on correct hand hygiene technique

4.2 PPE:

- ✓ PPE provided based on risk assessment (gloves, aprons, masks, visors); safe donning/doffing procedures

4.3 Cleaning & Disinfection:

- ✓ Daily cleaning with increased frequency during outbreaks; approved disinfectants in high-touch areas

4.4 Waste & Laundry:

- ✓ Segregation of clinical and domestic waste; secure sharps bins; separate soiled/clean linen handling

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SECTIONS 5-6: TRAINING & CLEANING PROTOCOLS

5 TRAINING

- ✓ **Induction:** All new staff complete IPC training including standard precautions, hand hygiene, PPE, waste disposal, and outbreak response
- ✓ **Refresher:** Every 2 years or earlier if policy changes or after outbreaks; tailored to staff roles
- ✓ **Records:** Attendance logged in Staff Training Register; PIC monitors compliance
- ✓ **Evaluation:** Training outcomes reviewed regularly and revised to align with national guidance

6 CLEANING PROTOCOLS

6.1 Daily Cleaning:

- ✓ General cleaning of all communal areas, bedrooms, bathrooms, kitchens, and corridors
- ✓ High-touch surfaces (door handles, switches, handrails) cleaned at least twice daily

6.2 Deep Cleaning:

- ✓ Weekly in high-traffic areas; more frequently during outbreaks
- ✓ Includes skirting boards, walls, under furniture, and sanitisation of soft furnishings

6.3 Standards:

- ✓ Only HSE-approved cleaning agents and disinfectants used; correct dilution and contact times
- ✓ Colour-coded equipment to prevent cross-contamination
- ✓ All cleaning activities logged with date, time, area, and staff initials; reviewed weekly by PIC

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SECTIONS 7-8: PPE & OUTBREAK RESPONSE

7 PPE & SUPPLIES

- ✓ **Types:** Gloves, aprons, surgical masks, FFP2/FFP3 respirators, face shields, goggles — worn per task-specific risk assessment
- ✓ **Storage:** Designated clean areas; stock levels checked weekly and reordered proactively
- ✓ **Supply Chain:** Infection control supplies in standard orders; emergency stock reserves maintained onsite
- ✓ **Monitoring:** PIC oversees stock levels and PPE compliance; misuse escalated immediately
- ✓ **Disposal:** Disposable PPE disposed per healthcare waste protocols; reusable items sanitised after each use

8 OUTBREAK RESPONSE

8.1 Identification:

- ✓ Two or more residents/staff with similar symptoms reported to PIC; clinical guidance sought from GP or HSE Public Health

8.2 Isolation & Control:

- ✓ Affected individuals isolated or cohorted; dedicated staff and equipment assigned
- ✓ Entry/exit protocols enforced; enhanced cleaning and additional PPE measures introduced
- ✓ Non-essential activities and visits suspended

8.3 Communication & Documentation:

- ✓ Families, staff, and health authorities informed with transparent updates throughout
- ✓ Full outbreak report maintained including timeline, actions, and outcomes
- ✓ Post-outbreak debriefing and review to inform future improvements

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SECTIONS 9–10: MONITORING & REVIEW

9 MONITORING & AUDIT

9.1 Routine Monitoring:

- ✓ Daily checks of hand hygiene facilities, PPE availability, and cleanliness
- ✓ PIC and team leaders conduct walk-through reviews and log compliance observations

9.2 Scheduled Audits:

- ✓ Formal IPC audits conducted monthly using a standard checklist
- ✓ Focus on cleaning protocols, equipment sanitation, staff compliance, and PPE usage
- ✓ Results reviewed by management during governance meetings

9.3 Outcomes & Actions:

- ✓ Findings documented, dated, and signed; action plans with assigned responsibility and deadlines
- ✓ Learning integrated into future training; residents and staff encouraged to provide feedback
- ✓ Results reported to Registered Provider and retained for HIQA inspection

10 POLICY REVIEW & CONTINUOUS IMPROVEMENT

- ✓ **Review Cycle:** Formally reviewed at least annually; interim reviews triggered by HIQA updates, outbreaks, legislation, or audit findings
- ✓ **Feedback:** Staff, residents, and families contribute via governance meetings; lessons from incidents and audits inform revisions
- ✓ **Version Control:** All versions dated, signed, and archived; most recent version clearly marked and accessible
- ✓ **Commitment:** Organisation aligned with best practice and regulatory expectations at all times